

Medicaid Innovation Accelerator Program (IAP)

Promoting Community Integration in Long-Term Services and Supports Programs—
Value-Based Payment for Home and Community-Based Services

December 19, 2018



Appendix

Category 2C: FFS with Link to Quality and Value – Pay for Performance

Approach	Example
Aggregate or per member per month (PMPM) bonus payment for meeting targets	Connecticut includes up to a \$500,000 performance incentive pool in its HCBS case management contract with five regional providers. Funds are allocated based on performance relative to peers.
Vary FFS payments based on performance rating	- Ohio pays an incentive payment to nursing facilities through a pool that is funded based on a withhold to the daily rate. Facilities can achieve up to five points, one for each of five quality measures. The quality pool is then allocated based on each facility's score. - The CMS Home Health Value-Based Purchasing Model is in nine states. Home health agencies will have payments adjusted based on improvements relative to the baseline year and performance relative to other home health agencies in the state. Maximum payment adjustment is 3% upward or downward, ramping up to 8% in five years.



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Category 3: APMs Built on FFS Architecture Gainsharing/Risk Sharing

Approach	Example
Performance bonus amount tied to total cost of care savings	- MaineHealth Accountable Care Organization contracts with Area Agency on Aging to provide care transition services; the AAA is eligible for shared savings of up to 5%. - Arizona health plan has attendant care-based incentive payment based on total cost of care.
Retroactive episode based payments for specific chronic conditions	Tennessee plans to implement 75 episodes of care in next five years. Some episodes (e.g., Pneumonia care, COPD) include opportunities for HCBS providers to partner with hospitals and post-acute care providers.
Case rates that include multiple provider types and/or services	- Nursing facility FFS rates that include primary care and pharmacy services. - IDD group home rates that include day programs, 24-hour care staffing and transportation services.



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Category 4: Population-Based Payment

Approach	Example
Population based payment for chronic conditions	New York has specialized Medicaid managed care programs focused on people with high behavioral health care needs and another focused on people with intellectual and developmental disabilities. The models include quality measures and withholds/incentives.
Sub-capitation or full risk capitation with link to quality	- Minnesota MLTSS plans with aligned D-SNPs sub-capitate providers for Medicare and MLTSS services and include downstream gain sharing with LTSS providers. - Oregon PACE – includes five measures in quality incentive program and pays a bonus for each measure met, up to \$100 pmpm.



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